

Opening Pandora's Box

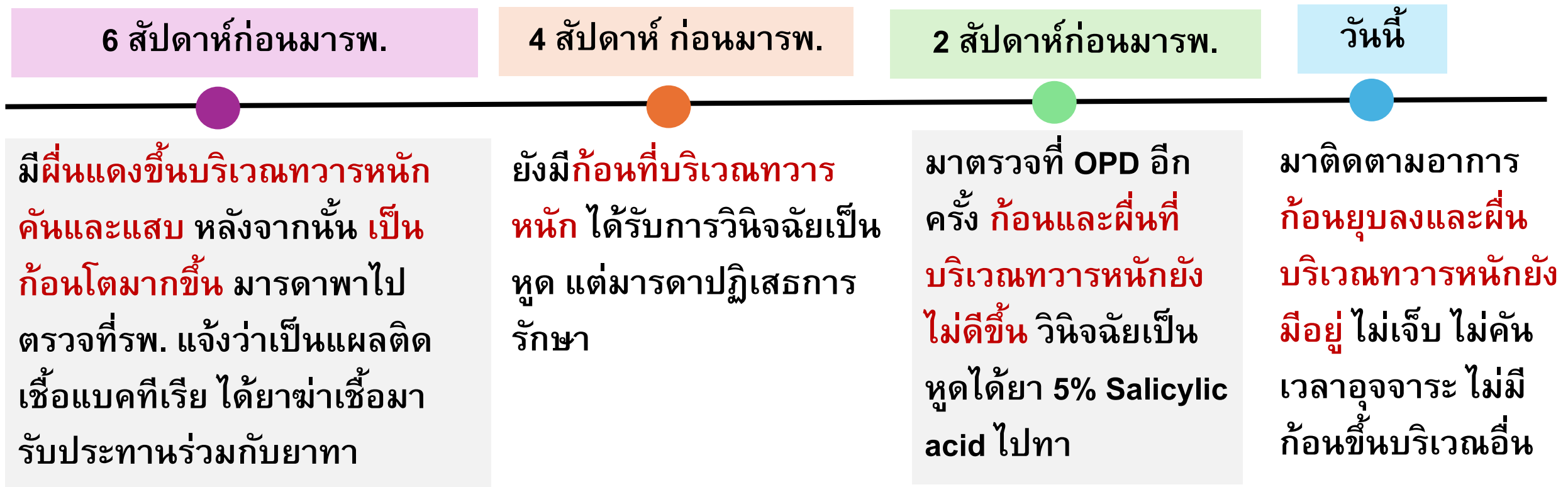
Natchaya Kunanitthaworn, M.D.

**Division of Infectious Disease, Department of Pediatrics
Faculty of Medicine, Chiang Mai University**



Case study

- Case boy 6 years old, no underlying disease
- Presented with **perianal rash and mass 6 weeks PTA**



Physical examination



What is the most likely diagnosis in this patient?

- A. Dermatitis
- B. Candidiasis
- C. Condyloma lata
- D. Condyloma acuminata

Physical examination



- No oral ulcer
- No rash on palms and soles
- Palpable left cervical lymph node 1 cm with painful

- Multiple erythematous moist nodules with greyish moist patch
- Erythematous papules and plaque with scale at perianal area

- Painful shallow ulcer at glans penis (painful urination)

Consult skin: Suspected condyloma lata

Investigations for diagnosis

Work up	Results
RPR	Reactive 1:64
TPHA	Reactive

Dx. Condyloma lata (Secondary syphilis)

RPR; Rapid Plasma Reagin

TPHA; Treponema pallidum Hemagglutination Assay

Syphilis

Etiology: *Treponema pallidum* subspecies *pallidum* (*Treponema pallidum*)

Stages	Timing	Clinical manifestations
Primary	10-90 days	- Painless or painful indurated ulcers (chancres) of the skin or mucous membranes at the site of inoculation. - Lesions appear on average, 3 weeks after exposure and heal spontaneously in a few weeks - Adjacent lymph nodes (large, not tender)
Secondary	1-3 months	- Fever, sore throat, muscle aches, rash, mucocutaneous lesions, hepatitis, and generalized lymphadenopathy - Generalized polymorphic maculopapular rash (palms and soles) - Condyloma lata (hypertrophic papular lesions) - Malaise, splenomegaly headache, alopecia, and arthralgia - Resolves spontaneously without treatment in approximately 3 to 12 weeks.

Syphilis

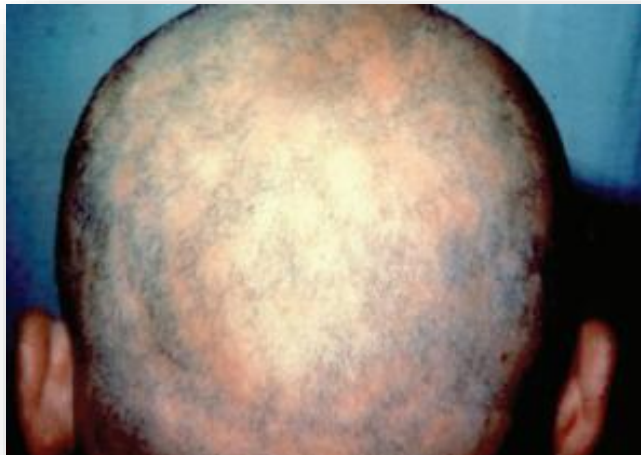
- **Latent syphilis** → Seroreactive but demonstrate **no clinical manifestations** of disease

Stages	Timing	Clinical manifestations
• Early latent	After primary or secondary stages, < 1 year of no symptoms	None
• Late latent	1 year in duration or The duration is unknown	None
• Tertiary	15 to 30 years after the initial infection	• Gumma formation (soft, noncancerous, granulomatous growths that can destroy tissue) • Cardiovascular involvement (aortitis)

Secondary syphilis



Condyloma lata



Moth-eaten alopecia

Other Sexually Transmitted Infections (STI) (1)

- Gonococcal infection (*N. gonorrhoeae*)
 - **Anorectal and pharyngeal infections** with *N. gonorrhoeae* are frequently **asymptomatic**
 - Investigations:
 - Culture for *N. gonorrhoeae*
 - NAAT for *N. gonorrhoeae* (from vaginal and urine specimens from girls and urine for boys)
 - Gram stains
 - Inadequate for evaluating prepubertal children for gonorrhea

Uncomplicated GC urethritis, proctitis (Among Infants and Children)		
	CDC, 2021	Thai, 2024
BW ≤45 kg	Ceftriaxone 25-50 mg/kg IV or IM, single dose (max 250 mg)	Ceftriaxone 50 mg/kg IV or IM, single dose (max 250 mg)
BW > 45 kg	Ceftriaxone 500 mg IM or IV single dose (As adult)	Ceftriaxone 500 mg IM or IV single dose

Other Sexually Transmitted Infections (STI) (2)

- Chlamydial Infection

(*C. trachomatis*)

- **NAATs for *C. trachomatis***

- Girls: vaginal and urine
- Boys: urine

Chlamydial infection (Among Infants and Children)		
	CDC, 2021	Thai, 2024
BW ≤45 kg	Erythromycin base or ethyl succinate for 14 days	Erythromycin base for 14 days
BW > 45 kg, < 8 yrs	Azithromycin 1 g oral, single dose	Azithromycin 1 g oral, single dose
≥ 8 yrs	• Azithromycin 1 g oral, single dose or • Doxycycline 100 mg oral bid for 7 days	• Doxycycline 100 mg oral bid for 7 days or • Azithromycin 1 g oral, single dose

Further investigations

- Work up other sexual transmitted disease (If available)
- Anti-HIV
- Anti-HBs, HBsAg
- Anti-HCV

Further investigations

Work up	Results
HBsAg	Negative
Anti-HBs	Negative 2.03
Anti-HCV	Negative

Work up	Result
Anti-HIV	Non-reactive

Urinalysis	Results
Squamous epithelium	0-1 HPF
WBC	None
RBC	1-2 HPF
Nitrite	Negative
Leukocyte esterase	Negative
Albumin	Trace

**What is the most appropriate management
for condyloma lata in this patient?**

- A. Cauterization**
- B. Benzathine Penicillin G (50,000 U/kg) IM single dose**
- C. Benzathine Penicillin G (50,000 U/kg) IM 3 single dose
at 1 wk-interval**
- D. Consult ID**

Treatment

Stages	Infants and children	Adults	Alternative treatment
Primary, Secondary, early latent syphilis	Benzathine penicillin G 50,000 units/kg body weight IM, up to the adult dose of 2.4 million units in a single dose	Benzathine penicillin G 2.4 million units IM in a single dose	<u>Penicillin allergy</u> - Doxycycline (100 mg orally 2 times/day for 14 days) - Tetracycline (500 mg orally 4 times/day for 14 days) - Azithromycin as a single 2-g oral dose
Late latent syphilis	-Benzathine penicillin G 7.2 million units total, administered as 3 doses of 2.4 million units IM each at 1-week intervals	Benzathine penicillin G 7.2 million units total, administered as 3 doses of 2.4 million units IM each at 1-week intervals	<u>Penicillin allergy</u> - Doxycycline (100 mg orally 2 times/day) - Tetracycline (500 mg orally 4 times/day each for 28 days)

Treatment in this patient

- Benzathine Penicillin G (50,000 U/kg) IM single dose
- Ceftriaxone 250 mg IM single dose
- Doxycycline 100 mg oral bid for 7 days

Common sexually transmitted infections for diagnosis and reporting of sexual abuse among infants and prepubertal children

TABLE 8. Implications of commonly encountered sexually transmitted or sexually associated infections for diagnosis and reporting of sexual abuse among infants and prepubertal children

Infection	Evidence for sexual abuse	Recommended action
Gonorrhea*	Diagnostic	Report [†]
Syphilis*	Diagnostic	Report [†]
HIV [§]	Diagnostic	Report [†]
<i>Chlamydia trachomatis</i> *	Diagnostic	Report [†]
<i>Trichomonas vaginalis</i> *	Diagnostic	Report [†]
Anogenital herpes	Suspicious	Consider report ^{†,¶}
Condylomata acuminata (anogenital warts)*	Suspicious	Consider report ^{†,¶,**}
Anogenital molluscum contagiosum	Inconclusive	Medical follow-up
Bacterial vaginosis	Inconclusive	Medical follow-up

Sources: Adapted from Kellogg N; American Academy of Pediatrics Committee on Child Abuse and Neglect. The evaluation of child abuse in children. Pediatrics 2005;116:506–12; Adams JA, Farst KJ, Kellogg ND. Interpretation of medical findings in suspected child abuse: an update for 2018. J Pediatr Adolesc Gynecol 2018;31:225–31.

* If unlikely to have been perinatally acquired and vertical transmission, which is rare, is excluded.

† Reports should be made to the local or state agency mandated to receive reports of suspected child abuse or neglect.

§ If unlikely to have been acquired perinatally or through transfusion.

¶ Unless a clear history of autoinoculation exists.

** Report if evidence exists to suspect abuse, including history, physical examination, or other identified infections. Lesions appearing for the first time in a child aged >5 years are more likely to have been caused by sexual transmission.

Further management in this patient

Sexual abuse?

Multidisciplinary Team:

- Consult G&D
- Consult pediatric psychiatrists
- Consult social worker

Consult G&D and pediatric psychiatrists

- ปัจจุบันเรียน อนุบาล 2 ชอบไปโรงเรียน
- ที่บ้านอยู่กัน 8 คน แม่ พ่อเลี้ยง ตัวเด็ก ตา ยาย ลุง พี่สะใภ้ และหลานชายอายุประมาณ 9-10 ขวบ
- ปกติชอบเล่นกับลูกชายลุงอายุ 9-10 ขวบ ชอบพากันไปเล่นน้ำด้วยกันที่คลองแม่ข่า
- มารดาบอกว่าคนในครอบครัวดูปกติดี ไม่มีเหตุการณ์ผิดปกติหรือ น่าสงสัยใดๆ ตัวเด็กเองก็ดูปกติดีมากับคุณแม่และพ่อเลี้ยง
- ปฏิเสธมีคนมาเล่นบริเวณอวัยวะเพศและทวารหนัก
- ผู้ป่วยยังมีความสุขดี ชอบไปโรงเรียน ชอบเล่นกับเพื่อน
- แม่ทำงานกลับดึกทุกวัน
- พ่อและแม่แยกทางกันตั้งแต่ผู้ป่วยอายุ 3 ปี (พ่ออายุ 26 ปี แม่อายุ 26 ปี)
- แม่ ทำงานถึงช่วงตี 4 – 2 ทุ่ม ทุกวัน ไม่มีวันหยุด
- พ่อเลี้ยงอายุ 20 ปี ทำงานช่วง 11 โมง – 3 ทุ่ม

Impression:

- Sexual abuse?
- Neglect child

Management:

- Counselling safety, sexual abuse prevention
- แนะนำเรื่องการเลี้ยงดู ความปลอดภัย การฝึกวินัย การจัดการอารมณ์
- ประสานสังคมสงเคราะห์
- ประสานครู

Other management: Sexual abuse

- Offer postexposure prophylaxis (PEP) for HIV infection
 - Children with **<72 Hours after sexual assault**
- Increased risk for future unsafe sexual practices
 - **High risk for HPV infection**
 - More likely to engage in these behaviors at an earlier age
 - ACIP recommends vaccination at **age ≥ 9 years** if they have not initiated or completed HPV vaccination

Back to our case

- Diagnosis: Condyloma lata (secondary syphilis)
- Management in this patient:
 - Benzathine Penicillin G (50,000 U/kg) IM single dose
 - Ceftriaxone 250 mg IM single dose
 - Doxycycline oral bid for 7 days
 - Post-exposure prophylaxis (PEP) for HIV infection → not indicated
 - Vaccination
 - Multidisciplinary team
 - Consult G&D, pediatric psychiatrists
 - Counselling safety, sexual abuse prevention
 - Social workers



Summary

Persistent genital or perianal lesions despite multiple visits require re-evaluation.

Condyloma lata in prepubertal children, especially perianal, is rare.

STI in a child should prompt **suspicion of sexual abuse** and activation of a multidisciplinary team.

One STI warrants screening for others.

Prevention focuses on child safety and sexual abuse prevention.

